DISCHARGE SUMMARY

Patient Name :	Mrs. DUNDAMMA	**IP No/Reg No/ UHID No :**	1261166/ASH--60336/UHID64944
D.O.D :	21/08/2026	**Sponsor :**	CASH
DOB :	07/08/1976	**Age/Sex :**	50Y 0M 11D/ Female
Mobile No :	8088103136	**Primary Dr. :**	Dr. ICU Team
DOA :	16/08/2026 18:09:21	**Primary Dr.Specialty :**	Doctor
Address :	NO 14 9TH CROSS OWAREGOWDA NAGAR R R NAGAR,Raja Rajeshwari Nagar, Bangalore, BANGALORE, Karnataka, India- 560098	**Admission BedNo :**	303

Final Diagnosis :

1. DIABETIC KETOACIDOSIS
2. DYSELECTROLYTEMIA → HYPOKALEMIA

Chief compliant

Complaints of drowsiness, arousable, obeying commands.
Complaints of generalized weakness (+).
History of S/P Diagnostic Laparoscopy with Adhesiolysis with suture repair of hernia on 13/08/2026.
No history of fever.
No drug / food allergies reported.

Past history

Known case of Type 2 DM on medications.

Family Medical History

Nothing significant.

Clinical examination at time of admission

- HR :127/min.
- BP : 140/80 mm Hg.
- SPO2 : 98% on room air.
- RR : 22/min.
- GRBS : 432 mg/dl.

Physical Examination

Moderately built & nourished.

Review Of System

CNS - Drowsy, arousable, obeying commands. GCS - 15/15.
CVS - S1 S2 (+), tachycardia (+).
RS - Bilateral NVBS (+).
PA - Soft.

Investigation

S.No.	Test Name	Result	UOM	Range Value
16/08/2026 7:27PM				
1	KETONE BODIES	PRESENT (+++)		
16/08/2026 9:06PM				
1	BLOOD CULTURE & SENSITIVITY	TRIDENT		
2	PROCALCITONIN-(PCT)	TRIDENT		
3	ABG	SUBMITED		

S.No.	Test Name	Result	UOM	Range Value
4	D-DIMER	TRIDENT	mg/l	< 0.5
16/08/2026 9:06PM : COMPLETE BLOOD COUNT (CBC):-				
1	HAEMOGLOBIN (HB)	16.00	g/dl	12 - 16
2	TOTAL COUNT (TC)	17500.00	cells/cumm	4000 - 11000
3	NEUTROPHILS	89.00	%	40 - 75
4	LYMPHOCYTES	7.00	%	20 - 45
5	EOSINOPHILS	1.00	%	1 - 6
6	MONOCYTES	3.00	%	2 - 10
7	BASOPHILS	0.00	%	0 - 1
8	RED BLOOD CELLS (RBC)	5.50	Millions/cumm	4.5 - 6.5
9	PLATELET COUNT	3.00	Lakh/Cumm	1.5 - 4.5
10	PACKED CELL VOLUME (PCV/HCT)	48.00	%	34 - 48
11	MEAN CELL VOLUME (MCV)	84.00	fl	80 - 96
12	MEAN CELL HAEMOGLOBIN (MCH)	30.00	pg	27 - 31
13	MEAN CELL HAEMOGLOBIN CONCENTRATE (MCHC)	35.00	g/dl	32 - 37
16/08/2026 9:06PM : RENAL FUNCTION TEST(RFT):-				
1	BLOOD UREA	29.00	mg/dl	10 - 40
2	SERUM CREATININE	0.60		0.6 - 1.10
3	URIC ACID	6.90	mg/dl	4.0 - 7.2
4	CALCIUM	8.50	mg/dl	8.5 - 11.0
5	Sodium Na+	143.00	mEq/L	135 - 146
6	POTASSIUM (K+)	4.20	mEq/L	3.5 - 5.5
7	CHLORIDE (Cl)	103.00	mEq/L	96 - 105
16/08/2026 9:06PM : LIVER FUNCTION TEST (LFT):-				
1	BILIRUBIN TOTAL	0.4		0.4 - 1.2
2	BILIRUBIN DIRECT	0.20	mg/dl	0.1 - 0.3
3	SGOT - AST	15.00	lU/ml	05 - 40
4	SGPT - ALT	13.00	lU/ml	05 - 40
5	ALKALINE PHOSPHATASE	207.00	IU/L	38 - 140
6	TOTAL PROTEIN	6.00	mg/dl	6.0 - 8.3
7	ALBUMIN	3.20	gm/dl	3.2 - 5.0
8	GLOBULIN	2.80	mg/dl	1.8 - 3.6
9	A/G RATIO	1.10		
16/08/2026 9:06PM : PROTHROMBIN TIME (PT/INR):-				
1	TEST	11.90		13 - 18
2	CONTROLE	11.70		
3	INR	1.00		
17/08/2026 12:21AM				
1	ABG	SUMBITTED		
17/08/2026 3:48AM				
1	POTASSIUM (K+)	3.30	mEq/L	3.5 - 5.5
2	ABG	SUMBITTED		
18/08/2026 7:54AM : SERUM ELECTROLYTES (NA,K,CL):-				
1	Sodium Na+	144.00	mEq/L	135 - 146
2	POTASSIUM (K+)	3.10	mEq/L	3.5 - 5.5
3	CHLORIDE (Cl)	106.00	mEq/L	96 - 105

Course In Hospitalization

The patient was admitted to ICU with above history and findings. Relevant investigations were done. All reports were enclosed. CBC - TC - 17,500 cells/cumm, LFT - Alkaline Phosphatase - 207 Iu/L, RFT - Normal, Ketone Bodies - Present (+++), Urine Routine - Protein - Traces, Sugar - 2.0%, 2-4 pus cells, 4-6 epithelial cells, Potassium - 3.30 mEq/L, USG Abdomen & Pelvis - Cholelithiasis / No evidence of cholecystitis / Non-obstructive right renal calculus / Partially distended urinary bladder with few free floating internal echoes and irregular mucosal wall thickening / Grade I fatty liver, D-Dimer - 1.34 mg/L, Procalcitonin - 0.370 ng/ml, S. Electrolytes - S. Sodium - 144 mEq/L / S. Potassium - 3.10 mEq/L / S. Chlorides

- 106 mEq/L. Patient was treated with Inj. Piptaz 4.5 gm IV TID, Inj. Metrogyl 100 ml IV TID, Inj. Paracetamol 1 gm IV TID, Inj. Pan 40 mg IV BD, Inj. Emeset 4 mg IV BD, Syp. Potklor 15 ml TID, Inj. FIASP according to sliding scale TID, Inj. Tresiba 10 units OD & other supportive care were given. ABG were regularly monitored. General Surgeon opinion was taken & advise followed. General Physician opinion was sought and advise followed. General surgeon review was done , Closely monitored GRBS levels Daily and Treated with Insulin ,Repeated electrolytes were done, Pottasium correction was given ,after stable condition patient shifted to ward on 18/08/26 ,continued same treatement .surgeon and Physician review done daily and advise followed, Patient sugar levels improved , Symptomaticaly better , The Patient General Condition Improved Gradually. The Patient Is Hemodynamically Stable And Discharged With Following Advice.

General Condition on Discharge

- HR : 72/min.
- BP : 130/70 mm Hg.
- SPO2 : 98% on room air.
- RR : 22/min.
- CVS : S1 S2 (+).
- RS : Bilateral air entry (+).
- PA : Soft, non tender.
- CNS : Conscious & oriented.

Discharge Medication

TAB ORNI'O' 1-0-1 X 5 DAYS(AF)
TAB SOMPRAZ 40MG 1-0-1 X 5 DAYS(BF)
TAB BUSCOPAN SOS FOR ABDOMINAL PAIN
SYP CREMAFFIN 10ML 0-0-1 X 5 DAYS(AF)
ABDOMINAL BINDER
TAB ISTAMET XR 100/1000 0-1-0 X TC(AF)
TAB GLYCOMET 500MG 1-0-1 X TC(AF)
ADVISE
MONITOR SUGAR LEVEL IN HOME DAILY
REPEAT FBS,HBA1C,PPBS AND URINE ROUTINE AFTER 3MONTHS

Follow-up Details

REVIEW AFTER 5 DAYS WITH DR SUJAY BR ON OPD BASIS WITH PRIOR APPOINMENT.
REVIEW AFTER 3 MONTHS WITH DR SHASHI KUMAR / DR VINOD ON OPD BASIS WITH PRIOR APPOINMENT WITH FBS,PPBS,HBA1C & URINE ROUTINE.

In case of any Emergency,please contact Casualty Medical Officer at Atreum Speciality Hospital

RESIDENT INCHARGE TYPED BY

Dr. ICU Team
(Doctor)
(DOCTOR)